HELIXONA

Building the Complex Chronic Program

The project, the patient journey, the people, and the meetings and workflows Kai will run to get it built. October 2026.

THE PROJECT

A nine-month program that needs its own platform

Helixona is an integrative medicine clinic in Irvine, California. Its flagship offer is the **Complex Chronic Program**: nine months of physician-led care for people with persistent fatigue, autoimmune conditions, Lyme, mold illness, long COVID and similar complex problems. A patient pays a separate assessment fee (Month 0) and then $2,000 a month for nine months, and may continue month to month afterwards if the physician and patient agree.

The clinic's medical record, **eClinicalWorks (eCW)**, stays the legal chart and the calendar for insurance visits. Everything the program needs beyond that is being built as a **patient platform**: website intake and qualification, the physician's questionnaire with e-signed consents, daily check-ins with alerts to the nurses, booking of rooms and devices, the membership agreement with recurring payments, and the emails and texts around all of it. A bot copies what matters back into eCW so nothing is entered twice.

The mockups of every screen already exist and are reviewed by the clinic owners in the browser. What is missing is the **operational definition**: for each step, who does what, with which rule, and what the patient sees. That is Kai's job: run the meetings, write the workflows down, and close the open questions so the build has nothing to guess.

- **Pilot first.** Three to five patients. One clinic. Everything else comes after the pilot works.
- **Clinical content belongs to Dr. D.** The questionnaire, check-ins and alert rules are implemented exactly as she supplies them; nobody rewrites a clinical question.
- **Configuration, not code.** Surveys, alert rules, modality rules, forms, message templates and lifecycle rules are edited by staff in admin screens, with versions and approvals.
- **Money never drives care.** A failed payment never discharges a patient or changes clinical care on its own.
- **Never "monitoring".** The platform is reviewed during office hours; every check-in says so and points to 911 for emergencies.

PATIENT JOURNEY

From website inquiry to the Month 9 review

1
Inquiry
Website form: Complex Chronic or Health Optimization

2
Screening call
Advisor qualifies: PPO, deposit, can come to Irvine

3
Intake
Questionnaire (6 forms), documents, 10 consents signed

4
Month 0
Assessment visit, separate fee, chart prepared

5
Plan of Care
Physician authorizes modalities and cadence

6
Program offered
Agreement signed, card or bank on file
GATED ON "PROGRAM OFFERED"

7
Active care
Bookings, daily check-ins, alerts, $2,000 / month × 9

8
Month 9 review
Continue · maintenance · graduate · withdraw · discharge

The agreement step is gated: a patient can only sign and set up payment once the physician has completed the Plan of Care and the registry shows "Program offered". Health Optimization patients follow a shorter path (their secure link arrives automatically, and they book provider visits and therapies without a membership). Anyone can also book a single therapy from the website without joining the program.

PEOPLE

Who decides what

Cassandra
CEO / PROGRAM ADMIN
Final say on scope, patient-facing language, dashboards.

Dr. D
MEDICAL DIRECTOR
Owns all clinical content: questionnaire, check-ins, alert rules, Plan of Care, Month 9 decisions.

Charlene
CHART PREP
Reviews intake, places notes in eCW, closes bot exceptions.

Karina

OPERATIONS AND FINANCE

Memberships, payments, failed-payment queue, booking exceptions.

Marie

NEW PATIENT COORDINATOR

Approves documents, schedules Month 0, patient communications.

Mira

FRONT DESK / APPOINTMENTS

Books insurance visits in eCW; treatment-only bookings.

Shibani

FORMS AND COMPLIANCE

Consents and legal forms.

Ana (advisor)

PATIENT ADVISOR

Screening calls, invitations, admissions outcome.

Carlos

PRODUCT AND BUILD

Mockups, platform build, eCW bot design.

Kai

WORKFLOWS AND MEETINGS

Runs the meetings below, documents each workflow, and builds the eCW bot flows.

MEETINGS

Fifteen meetings to define the program

Each meeting has one owner, one purpose and one output that goes straight into the platform configuration or the workflow document. Most take 45 to 60 minutes. Kai prepares the questions, runs the meeting, and writes the decision down the same day.

M1 Kickoff and scope

With: Cassandra (CEO), Carlos, Kai

Confirm the pilot (3–5 patients), the module order (intake first), who decides what, meeting cadence, and the weekly review.

> **Output:** Signed-off scope and decision owners; meeting calendar.

M2 Website intake and qualification

With: Cassandra, Marie (new patient coordinator), Mira

The two doors on the website, the three qualifying questions, what the screening call asks, who calls and when, how a non-candidate is handled.

> **Output:** Inquiry form content; screening call script; inquiry statuses and owners.

M3 Clinical questionnaire publish

With: Dr. D (Medical Director), Carlos

Walk through the six Complex Chronic forms and the Health Optimization forms exactly as supplied; confirm added diet options; SF-36 every six months; what is required vs optional.

> **Output:** Dr. D publishes Questionnaire v1 in the platform.

M4 Consents and legal forms

With: Shibani, attorney, Cassandra

The ten consents: who countersigns, which are signed at intake vs first visit, annual renewal, Privacy Officer contact, arbitration wording.

> **Output:** Final consent set and signing rules.

M5 Chart prep with Charlene

With: Charlene, Kai

Screen share: where the intake note goes in eCW, which document category, what a clean note looks like, how she reviews. Also the eCW bot flows J1 and J2.

> **Output:** Note template approved; eCW placement recorded; J2 ready to build.

M6 Month 0 and scheduling

With: Marie, Karina, front desk

How the assessment visit is booked after documents are approved, Month 0 fee timing (at booking or day-of), card on file, what the patient sees.

> **Output:** Month 0 workflow; patient home copy.

M7 Daily check-in, flare reports and alerts

With: Dr. D, nursing lead

Final wording of the daily check-in and the crash/flare report; alert levels, who receives each, response targets, escalation, after-hours language, the “not monitored” notice.

> **Output:** Survey v1 and alert rules v1 published.

M8 Modalities and Plan of Care

With: Dr. D, nursing, operations

List of modalities and the rooms/devices behind them, spacing and frequency rules, preparation instructions, which need physician clearance (EBOO, Erchonia laser), follow-up check-in timing.

> **Output:** Modality configuration v1.

M9 Treatment-only booking and pricing

With: Cassandra, Karina, front desk

Therapies anyone can book, prices and durations, group bookings (salt room), cancellation and no-show policy, confirmation message content.

> **Output:** Public booking catalogue and policies.

M10 Membership, payments and Stripe

With: Karina, attorney, Cassandra

$2,000 × 9 anniversary billing, Month 0 fee, failed-payment retries and grace period, holds (never automatic), refunds, Stripe account setup, leaving RepeatMD.

> **Output:** Payment rules v1; Stripe account live.

M11 Patient communications

With: Cassandra, Marie

Every email/text the patient gets (invitation, reminders, confirmations, receipts, payment notices), tone, timing, stop rules. Cassandra approves patient-facing language.

> **Output:** Message templates v1.

M12 Membership lifecycle and Month 9

With: Dr. D, Karina, Cassandra

Status list and reasons (pause, withdraw, discharge), the Month 9 review meeting, continue month-to-month vs maintenance vs graduate.

> **Output:** Lifecycle rules v1.

M13 eCW bots: calendar flows

With: Charlene, front desk, Kai

J3, J4, J5: how bookings are mirrored, what the morning reconciliation reports, how staff resolve exceptions. Which eCW client, rooms/devices as resources, visit types.

> **Output:** J3–J5 ready to build; eCW mapping table.

M14 Dashboards and reports

With: Cassandra, Karina

What each person checks daily and weekly: intake completion, chart prep, alerts, booking exceptions, payments, active members, Month 9 reviews, pilot defects.

> **Output:** Dashboard list v1.

M15 Pilot readiness

With: Everyone

Pick the first 3–5 patients, dry run the whole journey with a test patient, defect log, weekly bot review continues.

> **Output:** Go / no-go for the pilot.

WORKFLOWS

Eleven workflows to write down

One page each: trigger, steps, who acts at each step, what the patient sees, what gets recorded (who, when, why), and what happens when something goes wrong. The mockup screens are the illustrations.

WORKFLOW	FROM → TO	OWNERS	DEFINED IN
Intake	Website inquiry → screening call → patient record + invitation → questionnaire, documents, consents → intake review → chart prep → placed in eCW	Advisor, Charlene	M2, M3, M4, M5
Admissions	Screening outcome: accepted for Month 0 / denied / fell off, with reasons; insurance check	Advisor, Marie	M2, M6
Month 0	Documents approved → patient notified → assessment booked (self or by phone) → Month 0 fee → visit → Plan of Care	Marie, Dr. D, Karina	M6, M8
Program offer and agreement	Plan of Care done → status “Program offered” → agreement signed → card/bank authorized → “Active member”	Karina, Dr. D	M10, M12

WORKFLOW	FROM → TO	OWNERS	DEFINED IN
Daily check-in and alerts	Patient check-in or flare report → rules → alert level → nurse queue → acknowledge → follow-up → resolve; escalation on timer	Nursing, Dr. D	M7
Session booking	Patient books an authorized modality → rules checked → room/device held → mirrored to eCW → confirmed; staff resolve conflicts	Front desk, nursing	M8, M13
Treatment-only booking	Anyone picks a therapy → pays → signs that therapy's consent → confirmed; clearance therapies route to "Become a patient"	Front desk, Karina	M9, M13
Payments	Monthly anniversary charge → receipt; failed payment → Karina's queue → patient notice → retries → hold only with staff approval	Karina	M10
Communications	Event → template → email/text → delivery log; stop rules	Marie, Cassandra	M11
Month 9 review	Review scheduled → physician + patient decision → status change with reason → billing adjusted	Dr. D, Karina	M12
eCW bots	J1 verify chart · J2 note + PDF · J3 booking mirror · J4 morning read · J5 reschedule/cancel	Charlene, front desk	M5, M13

SEQUENCE

Six weeks of meetings

Week 1	M1 Kickoff · M2 Website intake · M3 Questionnaire publish
Week 2	M4 Consents · M5 Chart prep with Charlene · M6 Month 0
Week 3	M7 Check-ins and alerts · M8 Modalities · M9 Treatment booking
Week 4	M10 Payments and Stripe · M11 Communications · M12 Lifecycle
Week 5	M13 eCW calendar flows · M14 Dashboards

Week 6	M15 Pilot readiness · first patients invited

The order follows the build: intake first, because a patient cannot start without it; then clinical check-ins and alerts; then booking, payments and communications; the eCW calendar flows and dashboards last, when the pieces they connect exist.

ALREADY DECIDED

Do not reopen

- No eCW interface is available; integration runs through the bot, with every write verified and failures shown to staff.
- The platform owns rooms, devices and program bookings; eCW receives a copy. Insurance visits stay in eCW.
- Stripe for all payments; the platform never stores card or bank numbers.
- Build by module, intake first.
- Patients never self-enroll in the program; the website form leads to a screening call for Complex Chronic and to an automatic link for Health Optimization.
- Month 0 is a separate charge; the membership is $2,000 × 9; continuation after Month 9 is a physician + patient decision.

OPEN QUESTIONS

What the meetings must close

- Who countersigns consents, which are signed at intake vs at the first visit, annual renewal, and the Privacy Officer contact. (M4)
- Exact eCW location for the intake note and the PDF category; web or desktop eCW client. (M5, M13)
- Whether rooms and devices exist in eCW as resources today. (M13)
- Payment rules: retry cadence, grace period, hold policy, refunds; eCW license clause on automated access. (M10)
- Prices, durations and no-show fee for treatment-only bookings; which therapies need clearance. (M9)
- Clinical publish of Questionnaire v1 and the first alert rules. (M3, M7)